



جوستاف روسي انترناشيونال - مصر (EPE) * طلب علاج أول مرة / متردد

إسم المريض ربيعاً : رشا فريد خلف احمد
المحافظة : الجيزة
الحى / المدينة : اوسيم
رقم البطاقة : 28806012113021 رقم التليفون : 01027365985
العنوان : اوسيم
العمر (تاريخ الميلاد) : 01-06-1988 المهنة : ربه منزل
تاريخ تسجيل الإستمارة : 28-09-2026

الإقرار

أقر انا الموقع أدناه انني لا اتمتع باي نظام علاج مثل:- التأمين الصحي-القوات المسلحة-الشرطة-الهيئات-المؤسسات او اي نظام يتعارض مع نظام العلاج علي نفقة الدولة- وسوف اتحمل كافة نفقات العلاج في حالة ثبوت غير ذلك
الإحصائي الاجتماعي
التوقيع: (-----) المقرر بما فيه
التوقيع: (-----)

التشخيص الإكلينيكي : سرطان بالثدي (مبادرة)
نتائج الأبحاث : (مرفق)
التشخيص النهائي : سرطان بالثدي (مبادرة)
خطة العلاج : أبحاث أورام ماموجرام وسونار علي الثديين [10888] قسم العلاج : الاورام
التعليق على التشخيص : -----

تقرير اللجنة الثلاثية المتخصصة

الاسم: د/محمد كيلاني
الاسم: د/ندى حسن
الاسم: د/نرمين رمضان
الوظيفة: -----
الوظيفة: -----
الوظيفة: -----
التوقيع: -----
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التوقيع: -----
أطلعت اللجنة علي لائحة العلاج الخاصة بالمجالس وتلتزم جهة العلاج بلائحة المجالس الطبية. كما تقرر اللجنة بان البيانات المدونة فى هذا النموذج صحيحة.
تعتمد صحة البيانات المرفقة
رئيس اللجنة ومدير المستشفى.
تعهد المستشفى أن لا يتم تنفيذ القرار إلا بعد مراجعة الملف الطبي للمريض وإعادة مناظرة المريض بالمستشفى قبل الصرف.

أعضاء اللجنة

د. / ----- د. / ----- د. / ----- د. / -----

قرار اللجنة

التشخيص النهائي : -----
خطة العلاج : -----
المستشفى : -----
قسم : -----
جهة العلاج : -----
نفقات : -----
ملاحظات : -----

ختم
شعار الجمهورية

مدير عام
مديرية الشؤون الصحية

رئيس
المجلس الطبي المتخصص

--

Name: رشا فريد خلف احمد National ID: 28806012113021	
--	---

تقرير طبي

السيد
تحية طبية وبعد,,

إيماء للطلب المقدم من	بشأن تقرير طبي للمريض
بقسم:	رقم طبي:

نتشرف بان نوضح فيما يلي تقريرنا عن حالته
--

تاريخ وساعة الدخول : -- --:-- yyyy-mm-dd	تاريخ وساعة الخروج : PM 01:17 05-07-2026
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وبعد الكشف والفحوصات والمتابعة وجد انه
--

المریضة تعاني من ورم غير حميد بالثدي و تحتاج الي Bilateral mammogram and breast ultrasound
--

التاريخ : 05-07-2026	الوقت : -- --:--
	الطبيب المعالج : د/شيماء لاشين

$$P_{\alpha} = \frac{1}{2} \frac{P_{\alpha-1}}{P_{\alpha-2}}$$

وہمہ مقلد

Page 10

23

100

مفتی محمد رفیع الدین صاحب المدینۃ العلمیۃ

$$T = 2\pi\sqrt{L/g} = 2\pi\sqrt{0.5/9.8} = 1.41 \text{ s}$$


HX8973352

Patient ID: 84268	Patient Name English: Rasha Farid Khalaf
عمر ٢٧ سنة Age:	2025/06/18 12:30:45 PM :Report Date

Women's Health Outreach Program
Contrast Enhanced Digital Mammography

Clinical summary:

- Status post right WLE, yet with no definite residual malignancy detected on final post-operative pathology.

Technique

- MLO and CC views (low energy & recombined post contrast)
- Targeted High resolution ultrasound

Detailed Findings:

- **Density:** ACR B
- **Background Uptake:** mild background enhancement.

Right breast

- **Low energy (non-contrast images) show:** upper outer quadrant scarring with parenchymal distortion.
- **Recombined (post-contrast) images show:** corresponding focal non-mass enhancement measuring 21 x 16 mm, being at about 18 mm from nipple.
- **Targeted Ultrasound** demonstrated UOQ and upper central hypoechoic scar with a 12 o'clock peri-areolar (1.6 from the nipple) detected internal vascularity on limited color Doppler examination.

Left breast

- **Low energy (non-contrast images) show:** upper central, middle third, equal density, oval mass lesion with partially obscured borders (more appreciated on CC views).
- **Recombined (post contrast) images show:** corresponding mild heterogeneous enhancement, measuring 10 x 8 mm, being at about 5 cm from nipple.
- **Targeted Ultrasound** demonstrated 12 o'clock circumscribed, oval shaped, isoechoic mass lesion measuring about 11 x 4 mm, being at about 3 cm from nipple.
- Normal skin/subcutaneous tissue and nipple/areolar complex.

Impression:

- **Right breast (BIRADS 4):** Upper central (operative bed) operative bed focal non mass enhancement and distortion ? post-operative changes ? suspicious lesion, for histopathological correlation.
- **Left breast (BIRADS 3):** 12 o'clock probably benign lesion mostly fibroadenoma, for US follow up in six months.

Much Obligated,
Dr. Sherihan Fakhry, MD.
Dr. Amal Hegazi, M.Sc.

جمهورية مصر العربية
بطاقة تحقيق الشخصية

رشا
فريد خلف احمد
٥٧ ش عبدالعال محمد همام - البراجيل
مركز اوسيم - الجيزة

٢٨٨٠٦٠١٢١١٣٠٢١

HX8873352

٢٠١٩/٠٣ ٢٨٨٠٦٠١٢١١٣٠٢١

ربة منزل
أنثى مسلمة
مستشفى ابراهيم عبدالفتاح
البطاقة سارية حتى ٢٠٢٦/٠٣/٠٦

١

١٢٩
٥٥

١٠٩٧٢٦٥٩ ٨٥

فريد خلف احمد
٢٠١٩



ERM036748066

٧٢٦٥٩١٢٥

٣٦

بطاقة



MDT FORM

4 / 8 / 2025 G

Name	رشا فريد فلفاح	MRN	٢٨٨٠٧٠١٢٨٢٠
		ID	139125
Age	36/6	Phone	010 ٢٧٣ ٦٠٩٨٠
Menopausal status	Peri	Marital status	
NRS SCORE		Family history	
BMI		REASON OF MDT	Post op
CO morbidity			

Clinical Data

BX: previous IDC

R

L

ER: +ve PR: +ve HER 2: +2 Ki67:

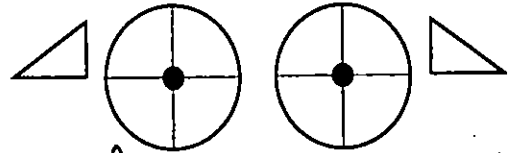
Sonomammography:

not amplified

CEM-af → RT focal Non mass enhancement

Mets Workup: 0 distalation

True → proliferative changes & microcalcifications
e' Surrounding PASH



TNM

Recommendations:

MDT OUTCOME & PLAN

- for Contrast guided wire localization
- w/E over wire

SIGNATURE

MEDICAL ONCOLOGY	SURGERY	RADIOLOGY	PATHOLOGY	Nutrition	Psychiatry

MDT FORM 2 / 7 / 2025 G

Name	شافريه خلفاً ح	MRN	139 / 50
Age	36yr	ID	588715112025
Menopausal status		Phone	0102770910
NRS SCORE		Marital status	
BMI		Family history	
CO morbidity		REASON OF MDT	post op

Clinical Data

BX:				R L TNM
ER:	PR:	HER 2:	Ki67:	
Sonomammography:				
Mets Workup: previous Bx IDC GII ER +ve PR +ve SIS H: not amplified.				

Recommendations: pt under went upfront WLG + SLN 3/2025
 But no tumor tissue detected on the specimen
 SLN 0/4

post op: C ESM : Rt Breast upper central op. bed. focal
 Non mass & distortion post op?? Susp. lesion

For possibility of (Bx) vs Surgical (Ex) WLG as the

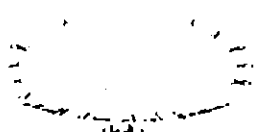
MDT OUTCOME & PLAN Biopsied mass was not excised in the previous surgery

For the cut biopsy (Bx) guided / ESM 2/7/2025

After Bx Proliferative changes micro

SIGNATURE

MEDICAL ONCOLOGY	SURGERY	RADIOLOGY	PATHOLOGY	Nutrition	Psychiatric
Dr. Hoss	Dr. Hoss	Dr. Hoss	Dr. Hoss	Dr. Hoss	Dr. Hoss



Presidential Program for Women's Health
Ministry of Health and Population

البرنامج الرئاسي لصحة المرأة
وزارة الصحة والسكان



لصحة المرأة

MDT form

A-Patient ID:

1- Name : رشا فريد خلف احمد

Sex:

3- Medical number:

Female

83254835

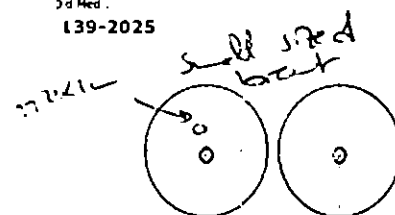
Old Med :

Did Med :

139-2025

2-ID number:

Patient Age : 37 Y



Family history

Family history

-ve

Phone

Phone

01027365985

BMI

34

REASON OF MDT

REASON OF MDT

New

marital status

Marital status

Married

NRS Score

NRS Score

GROUP

GROUP

Date of MDT

Date of MDT

12/02/2025 ☐

Co morbidity

Co morbidity

-ve

Menopausal status

Menopausal status

Initial STAGE

Initial STAGE

Clinical data

Breast Imaging :

rt breast mass at 1 oclock 1x1 cm
non specific axillary LN BIRADS 4

Mets Workup :

cap:free

BX IDC RX COMMENT	ER 8/8 LR COMMENT	PR 8/8 LR COMMENT	Her 2 Neu 2 RX COMMENT	SIS-1 Not Amplified SISX COMMENT	Ki67 5
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T 1	N 0	M
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Sonomammography

PATIENT HISTORY

MDT OUTCOME & PLAN

MDT OUTCOME & PLAN

for new BSM then through the clinic IF PROVED T1N0 for upfront rt WLE& SLNB (SKIN MARK)

SIGNATURE			
MEDICAL ONCOLOGY	RADIOLOGY	RADIOLOGY	
SURGERY	PATHOLOGY	PATHOLOGY	
Psychiatric	Nutrition	Nutrition	



مبادرة رئيس الجمهورية
لدعم صحة المرأة المصرية

الاستشارة
طبية
مصر

MDT FORM

12/5 /2025 G

Name	د. محمد حجاز	MRN	13912025
Age	47	ID	
Menopausal status		Phone	01027365985
NRS SCORE		Marital status	Married
BMI		Family history	-ve
CO morbidity	-ve	REASON OF MDT	New Case

Clinical Data

BX: Res ^D IDC	R	L	
ER: 818	PR: 818	HER 2: +2	Ki67: 5%
Sonomammography: Not available			
Right Breast mass at 11 o'clock 1x2cm - Non-specific Axillary lvs. (US 11A054)			
Mets Workup: at CAP → free			
T N M			

Recommendations:

MDT OUTCOME & PLAN

For - upfront Rt skin mark and WLE + SLNB
(after new mammography to be reviewed
at surgery clinic).

SIGNATURE

MEDICAL ONCOLOGY	SURGERY	RADIOLOGY	PATHOLOGY	Nutrition	Psychiatric
د. محمد حجاز					



Patient Name: Rasha Farid Khalaf Ahmed	Date : 01/03/2025
Exam : ultrasound BREAST (BOTH SIDE)	Age : 36 Year(s)
Medical ID: 139/2025	Pat. ID: 96916

**BILATERAL BREAST MAMMOGRAPHY AND HIGH RESOLUTION
ULTRASOUND**

Indication:

Right breast pathologically proven invasive duct carcinoma.

Technique

- Bilateral MLO and CC views
- Bilateral breast ultrasound

Report:

- ACR:C.
- Right breast UIQ small focal asymmetry with parenchymal distortion about 4 cm from the nipple corresponding to 12 o'clock small irregular hypoechoic lesion measuring 1.3X1.7 cm.
- No left suspicious mass lesions, suspicious micro calcifications or architectural distortion: detected.
- no left solid or cystic mass lesions.
- Normal skin thickness and nipple-areola complex.
- Nonspecific axillary LNs bilaterally.
- No right infra or supraclavicular lymph nodes.

Opinion:

- Right breast UIQ pathologically proven malignant mass lesion (BIRADS 6)

Yours truly

Signature Dr. Youmna tarek

01/03/2025 1:17 PM

Name: Rasha Fareed Khalaf
Date: 20/03/2025

37Y

F

Ultrasound guided skin mark:

- *Ultrasound guided skin mark was placed (x) over the lesion seen in the right breast at 11-12 o'clock about 5.9 mm from the skin.*

Best Regards,

***Dr. Ahmed M. Rozeka, Msc
Specialist Radiologist***

Patient ID: 86702	Patient Name English: Rasha Farid Khalaf Ahmed
Age: ٣٧ سنة, ٢ شهر	Report Date :2025/08/13 10:04:29 AM

Ultrasound Guided Wire localization:

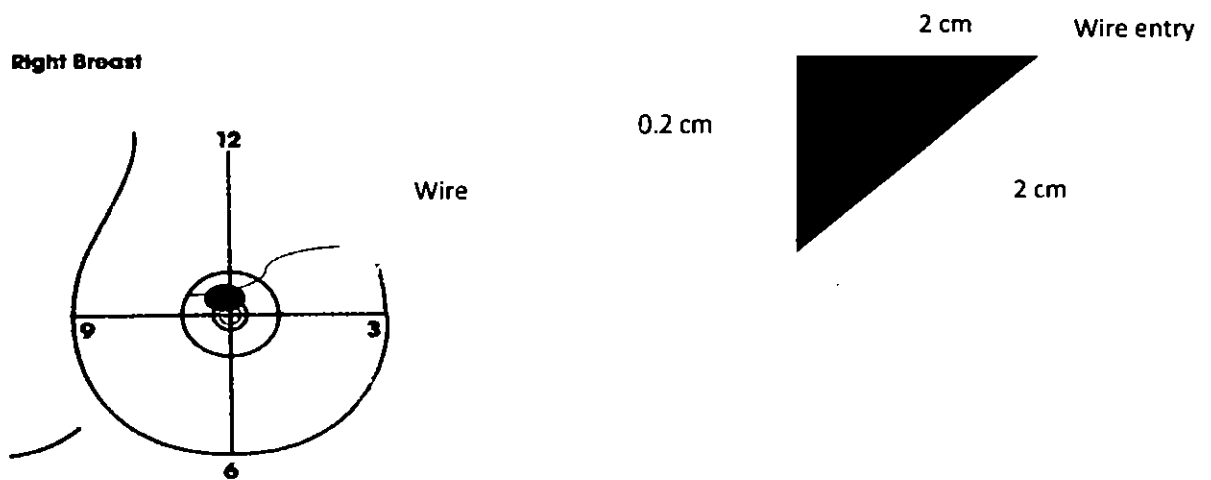
Clinical summary: Pathologically proven right breast proliferative changes with surrounding PASH, for pre-operative wire localization.

Technique:

Under complete sterile conditions, 3ml of local anaesthetic were administered at the site of concern in the RT breast. One 21G wire was used to localize the lesion.

The wire is located at 1:00, para-areolar region.

The wire was inserted transversely from the inner aspect for about 2 cm, being below the skin by 0.2 cm to reach the lesion with the hook is transfixing the lesion.



Much Obligated
Dr.Sherihan Fakhry,MD
Dr. Reham Rady,FFBR, FEBR

Patient ID: 84268	Patient Name English: Rasha Farid Khalaf
Age: ٢٧	2025/06/18 12:30:45 PM :Report Date

Women's Health Outreach Program
Contrast Enhanced Digital Mammography

Clinical summary:

- Status post right WLE, yet with no definite residual malignancy detected on final post-operative pathology.

Technique

- MLO and CC views (low energy & recombined post contrast)
- Targeted High resolution ultrasound

Detailed Findings:

- **Density:** ACR B
- **Background Uptake:** mild background enhancement.

Right breast

- **Low energy (non-contrast images) show:** upper outer quadrant scarring with parenchymal distortion.
- **Recombined (post-contrast) images show:** corresponding focal non-mass enhancement measuring 21 x 16 mm, being at about 18 mm from nipple.
- **Targeted Ultrasound** demonstrated UOQ and upper central hypoechoic scar with a 12 o'clock peri-areolar (1.6 from the nipple) detected internal vascularity on limited color Doppler examination.

Left breast

- **Low energy (non-contrast images) show:** upper central, middle third, equal density, oval mass lesion with partially obscured borders (more appreciated on CC views).
- **Recombined (post contrast) images show:** corresponding mild heterogeneous enhancement, measuring 10 x 8 mm, being at about 5 cm from nipple.
- **Targeted Ultrasound** demonstrated 12 o'clock circumscribed, oval shaped, isoechoic mass lesion measuring about 11 x 4 mm, being at about 3 cm from nipple.
- Normal skin/subcutaneous tissue and nipple/areolar complex.

Impression:

- **Right breast (BIRADS 4):** Upper central (operative bed) operative bed focal non mass enhancement and distortion ? post-operative changes ? suspicious lesion, for histopathological correlation.
- **Left breast (BIRADS 3):** 12 o'clock probably benign lesion mostly fibroadenoma, for US follow up in six months.

Much Obligated,
Dr. Sherihan Fakhry, MD.
Dr. Amal Hegazi, M.Sc.



Patient Name: Rasha Farid Khalaf Ahmed	Date : 16/01/2025
Exam : C.T CHEST WITH CONTRAST	Age : 36 Year(s)
Medical ID: 139/2025	Pat. ID: 96916

POST-CONTRAST MULTI-SLICE CT SCAN OF THE CHEST, ABDOMEN AND PELVIS:

- **Reported case of right breast cancer.**

I Chest:

- No sizable pulmonary nodules, masses, consolidation or cavitary lesions.
- No CT evidence of pathological mediastinal lymph node enlargement.
- No pleural or pericardial collection, pleural thickening or masses.
- No cardiac gross enlargement.

II Abdomen & pelvis:

- Average sized liver showing homogenous parenchymal attenuation. No sizable focal lesions.
- No intra-hepatic or extra-hepatic biliary radicles dilatation.
- Normal CT appearance of the spleen, pancreas, kidneys, suprarenal glands, aorta and IVC.
- Averagely filled urinary bladder showing no sizable diverticular outpouching.
- No pathological abdominal or pelvic lymph node enlargement.
- No ascites.
- No sizable worrisome osseous lesions.

Opinion:

Reported case of right breast cancer revealed:

- **No sizable pulmonary or hepatic lesions.**

Dr. Mohamed Mohsen, M.D.

Name: Rasha Fareid

Date: 19-12-2024

**BILATERAL BREAST MAMMOGRAM &
HIGH RESOLUTION ULTRASOUND**

- Bilateral scattered fibro-glandular density (ACR b).
- No breast suspicious mass lesions can be detected.
- No suspicious micro-calcification or architectural distortion detected.

Complementary ultrasound showed:

- Right breast UOQ hypoechoic irregular solid mass lesion with speculation with minimal central vascularity, seen in 11 o'clock, measuring 1x1cm.
- No left breast solid or cystic mass lesions.
- Normal left breast skin thickness, nipple areola complex.
- Bilateral Non-specific left axillary LNs.

Opinion:

- Right breast UOQ 11 o'clock solid mass lesion (BIRADS 4)

Dr/Bossy Sayed

Patient Name: Aameena Kamel Mahmoud Youssef	Date : 23/06/2025
Exam : MAMMOGRAPHY BOTH	Age : 52 Year(s)
Medical ID: 9931	Fat. ID: 100198

**DIGITAL BILATERAL BREAST MAMMOGRAPHY &
COMPLEMENTARY ULTRASOUND**

INDICATION:

Patient is 52yrs old and is complaining of a left breast palpable lump

REPORT:

- Scattered fibro glandular density, (ACR B).
- Multiple circumscribed rounded lesions are seen in the left UOQ, corresponding to the site of clinical complaint, complimentary U/S examination depicted complex cystic and solid lesions, located at 2-3 o'clock, para-areolarly, with the largest measuring 3x2cm.
- No suspicious micro calcifications or architectural distortion detected.
- Normal skin thickness, and nipple-areola complex.
- Bilateral nonspecific axillary nodes are noted.

OPINION:

- Left breast UOQ complex cystic lesions, (BIRADS 4a), for biopsy to exclude any underlying cellular activity.
- Normal study of the right breast, (BIRADS 1).

**Best regards,
Dr. Mahi Mokhtar**



Patient Name	: الأستاذ / رشا فريد خلف احمد	Request No.	: 13J08015
Patient No.	: 28806012113021	Request Date	: 23/12/2024
Path No.	: 5168/24	Verification Date	: 03/02/2025
Sex	: Female	Financial Category	: اورام دار السلام - نفقة الدولة
Age	: 36 Y	Department	: Out Patient
Referring Doc	:	Registered by	: Mrs.Mayada Hegazy

Pathology Report

Clinical Data:

Right breast mass.

Nature of Specimen:

Tru cut biopsy collected at 23/12/2024.

Technique:

- Sections were prepared from the paraffin block, processed in Bench Mark ULTRA machine (Ventana) and stained by rabbit monoclonal antibodies using ultra view, HRP DAB as chromogen and hematoxylin as counter stain. The sections treated against:
- SISH.

Results:

* IPT :

- SISH: Not amplified, average HER2 copy number: 3.2, HER2/CEP17: 1.4.

Conclusion:

Right breast mass, tru cut biopsy:

- Invasive duct carcinoma grade II.

IPT:

- SISH: Not amplified.

Dr.Walaa Ghoneim

Dr.Esraa Magdy

Verified by :



Patient Name	: الأستاذة / رشا فريد خلف احمد	Request No.	: 13001398
Patient No.	: 28806012113021	Request Date	: 20/03/2025
Path No.	: 1111/25	Verification Date	: 16/04/2025
Sex	: Female	Financial Category	: نفقة الدولة - اورام دار السلام
Age	: 36 Y	Department	: Out Patient
Referring Doc :		Registered by	: Ali mostafa

Pathology Report

- Free resection margins.
- No lymphatic tumor emboli
- No Perineural invasion.
- Reactive lymph nodes (0/4)

Comment:

- No residual tumor tissue detected in specimen received. The tumor may be totally submitted in the previous biopsy.
- Case for mandatory correlation with clinical & radiological findings for proper assessment.

Signature

Mayada Ghonaim

Dr. Marwa Mahmoud

Verified by :

Printing Date : 16/04/2025 12:27:29

Page 2 of 2

Printing By : Mrs.Mayada Hegazy

CAP
ACCREDITED

غرفة رقم (4) - الدور الأول - مبنى رقم (987) - كورنيش النيل - مصر القديمة - القاهرة - مصر
Room No. (4) - First Floor-Building No. 987- Nile Corniche - Misr El Qadema - Cairo, Egypt

Patient Name : الأستاذ / رشا فريد خلف احمد	Request No. : 13001398
Patient No. : 28806012113021	Request Date : 20/03/2025
Path No. : 1111/25	Verification Date : 16/04/2025
Sex : Female	Financial Category : نفقة الدولة - اورام دار السلام
Age : 36 Y	Department : Out Patient
Referring Doc :	Registered by : Ali mostafa

Pathology Report

Clinical Data: Right breast mass, Patient has a previous core biopsy pathology report (5168/24), diagnosed as IDC, GII.

Nature of the specimen: Right WLE & SLNs

Gross Examination:

Two specimens were received submitted as:

1- Right WLE measuring 5x4 cm, Cut section revealed an ill-defined rubbery grayish white mass measuring 2.5x2.5x2 cm, located 0.5 cm from least resection margin (deep).

Also there was another rubbery grayish white areas measured 3x1 cm & 3x2 cm.

2- Sentinel lymph node: 4 nodules ranging in size up to 2x1.5 cm.

• Frozen section:

Free Margins.

Free SLNs (0/4).

Microscopic examination:

• No residual tumor tissue detected.

• Angioinvasion: No

• Ductal carcinoma in situ: No

• Skin involvement: No

• Lymph node involvement: No (0/4)

• Resection Margins: Free margins.

• Histologic response to therapy: N/A

• Additional pathologic findings: Fibroadenosis & Fibrotic stroma.

Diagnosis:

Right breast cancer, Right WLE & SLNs;

- No residual tumor tissue detected.

Verified by :



Patient Name :	الأستاذة / امينه كامل محمود محمد	Request No. :	13003670
Patient No. :	27306062100161	Request Date :	30/06/2025
Path No. :	2174/25	Verification Date :	08/07/2025
Sex :	Female	Financial Category :	اورام دار السلام - مبادره
Age :	52 Y	Department :	Out Patient
Referreing Doc :		Registered by :	.marwa mohamed youss

Pathology Report

Clinical Data: Left breast para-areolar 2-3 O'clock lesion.

Nature Of Specimen: Tru-cut biopsy, collected at 29/6/2025.

Gross Examination:

Multiple grayish white tissue cores up to 0.5 cm long, totally submitted.

- Sections prepared from paraffin block for immune-staining using Bench Mark ULTRA system for; CALPONIN clone (EP798Y), ER clone (1E2).

Microscopic Examination:

Section examined reveals cores of breast tissue showing dilated mammary duct with hyperplastic epithelium which is thrown into folds to form compound polypoid structures with no identified myoepithelial layer within the submitted core.

IPT Results:

- Calponin: Negative.
- ER: Positive.

Diagnosis:

Left breast lesion, Tru-cut biopsy:

Papillary breast lesion, for total excision.

- Differential diagnosis includes (Atypical ductal hyperplasia, Papillary DCIS, Encapsulated papillary carcinoma, Invasive papillary carcinoma).

Signature

Dr. Walaa Ibrahim Ghoneim.

Dr. Heba Hassan Adly.

Verified by :

Dr. Duaa Mohamed

Printing Date : 08/07/2025 09:25:09

Page: 1 of 2

Printing By : Mrs. Mayada Hegazy



غرفة رقم (4) - الدور الأول - مبنى رقم (987) - كورنيش النيل - مصر القديمة - القاهرة - مصر
Room: No. (4) - First Floor - Building No. 987 - Nile Corniche - Misr El Qadema - Cairo, Egypt

Patient Name	: الأستاذ / رشا فريد خلف احمد	Request No.	: 13002299
Patient No.	: 28806012113021	Request Date	: 14/08/2025
Path No.	: 2540/25	Verification Date	: 16/08/2025
Sex	: Female	Financial Category	: جوستاف روسي - نفقة الدولة
Age	: 37 Y	Department	: Out Patient
Referring Doc :		Registered by	: Ali mcstafa

Pathology Report

Clinical data

Right breast lesion.

Nature of specimen

WLE specimen.

Gross examination

Wired single fibrofatty tissue piece measured 3.5x2.5x1.2 cm, skin ellipse measured 3x0.6 cm, serial cuts revealed rubbery grey tan and pink mass measured 2.5x1.3x1.1 cm, away from least inferior margin by 0.1 cm.

Microscopic examination

Microscopic examination revealed breast tissue showing diffuse stromal fibrosis and focal adenosis. Some of the proliferating ducts are irregularly dilated. The ducts are lined by inner columnar and outer myoepithelial cells with associated PASH. No atypia or malignancy in sections examined.

Diagnosis

Right breast lesion, WLE:

Picture compatible with fibroadenosis with associated PASH (Pseudo-angiomatous stromal hyperplasia). No malignancy.

Signature

Prof. Dr. Gamal Abdel Aty

Dr. Heba El-Shafeey.

Verified by :

Dr. Heba Hasan



Patient Name	: الأستاذ / رشا فريد خلف احمد	Request No.	: 13002299
Patient No.	: 28806012113021	Request Date	: 14/08/2025
Path No.	: 2540/25	Verification Date	: 16/08/2025
Sex	: Female	Financial Category	: جوستاف روسي - نفقة الدولة
Age	: 37 Y	Department	: Out Patient
Referring Doc :		Registered by	: Ali mostafa

Pathology Report

Verified by :
Dr.Heba Hasan

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Page 2 of 2

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غرفة رقم (4) - الدور الأول - مبنى رقم (987) - كورنيش النيل - مصر القديمة - القاهرة - مصر
Room No. (4) - First Floor-Building No. 987- Nile Corniche - Misr El Qadema - Cairo, Egypt

Patient Name	: الأستاذ / رشا فريد خلف احمد	Request No.	: 13005356
Patient No.	: 28806012113021	Request Date	: 14/07/2025
Path No.	: 2258/25	Verification Date	: 21/07/2025
Sex	: Female	Financial Category	: جوستاف روسي - نفقة الدولة
Age	: 37 Y	Department	: Out Patient
Referring Doc	:	Registered by	: Mrs.Mayada Hegazy

Pathology Report

Clinical data

Right breast 12 o'clock lesion , at operative bed of post WLE

Nature of specimen

Tru cut biopsy, collected on 13/7/2025

Gross examination

Multiple(3) grayish white tissue cores up to 0.7 cm, totally submitted.

Technique:

- Sections were prepared from the paraffin block, processed in Bench Mark ULTRA machine (Ventana) and stained by rabbit monoclonal antibodies using ultra view HRP DAB as chromogen and hematoxylin as counter stain. The sections treated against:
ER clone (SP1)

Microscopic examination

Examination showed ducts showing bland proliferative ductal epithelial cells with preserved myo-epithelial layer .There are areas of microcalcifications with surrounding breast tissue showing normal breast tissue showing complex interanastomosing spaces in dense collagenous keloid-like stroma . No mitotic figures, no necrosis, no atypia or malignancy.

IPT :

ER: Heterogenous pattern

Diagnosis

Right breast 12 o'clock lesion, Tru cut biopsy:

Verified by :

Dr.Heba Hasan



Patient Name	: الأستاذ / رشا فريد خلف احمد	Request No.	: 13005656
Patient No.	: 28806012113021	Request Date	: 14/07/2025
Path No.	: 2258/25	Verification Date	: 21/07/2025
Sex	: Female	Financial Category	: جوسفات روسي - نفقة الدولة
Age	: 37 Y	Department	: Out Patient
Referreing Doc :		Registered by	: Mrs.Mayada Hegazy

Pathology Report

Proliferative changes & microcalcifications with surrounding PASH(pseudo- angiomatous stromal hyperplasia)

Signature

Prof. Dr. Maha Guimei

Dr.Walaa Ibrahim Ghoneim

Verified by :

Dr.Heba Hasan

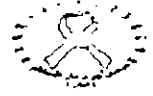
Printing Date : 26/07/2025 09:24:08

Page 2 of 2

Printing By : .marwa mohamed youss



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Room No. (4) - First Floor-Building No. 937- Nile Corniche - Misr El Qadema - Cairo, Egypt



Patient Name : الأستاذة / رشا فريد خلف احمد	Request No. : 13008015
Patient No. : 28806012113021	Request Date : 23/12/2024
Path No. : 5168/24	Verification Date : 01/01/2025
Sex : Female	Financial Category : نفقة الدولة - اورام دار السلام
Age : 36 Y	Department : Out Patient
Referring Doc :	Registered by : Mrs.Mayada Hegazy

Pathology Report

Clinical Data: Right breast mass.

Nature of Specimen: Tru-cut biopsy, collected at 23/12/2024

Gross Examination:

- Fragmented grayish white tissue cores up to 0.5 cm, totally submitted.

Microscopic Examination:

- * Histological type: Invasive duct carcinoma.
- * Grade (differentiation+ nuclear pleomorphism+ mitosis): (3+2+1) Grade II.
- * Ductal carcinoma in situ: Not detected
- * Perineural Invasion: Not detected
- * Angioinvasion: Not detected

Diagnosis:

Right breast mass, Tru-cut biopsy:

Invasive duct carcinoma, Grade II.

For IPT (ER, PR, Her2 and Ki67).

Signature

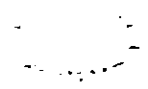
Prof. Dr. Lubna Omar

Dr. Menna Nabil

Verified by :

dr.esraa magdy





Patient Name	الأستاذة / رشا فريد خلف احمد	Request No.	13008015
Patient No.	28806012113021	Request Date	23/12/2024
Path No.	5168/24	Verification Date	29/01/2025
Sex	Female	Financial Category	اورام دار السلام - نفقة الدولة
Age	36 Y	Department	Out Patient
Referreing Doc.			
		Registered by	Mrs.Mayada Hegazy

IMMUNOHISTOCHEMISTRY REPORT

no staining; 1, >1%; 2, between 1% and 10%; 3, between 11% and 33%; 4, between 34% and 66% and 5, between 67%-100% of the cells staining). Intensity of positive tumor cells (0, none; 1, weak, 2, intermediate; and 3, strong). Total score from 0 to 8.

* Scoring system: "Her2"

Reference: ASCO-CAP Her2 Test Guideline Recommendations, 2013. IHC 0: as defined by no staining observed or membrane staining that is incomplete and is faint/barely perceptible and within < 10% of the invasive tumor cells. IHC 1+: as defined by incomplete membrane staining that is faint/barely perceptible and within < 10% of the invasive tumor cells. IHC 2+: based on circumferential membrane staining that is incomplete and/or weak/moderate and within < 10% of the invasive tumor cells or complete and circumferential membrane staining that is intense and within < 10% of the invasive tumor cells. IHC 3+: based on circumferential membrane staining that is incomplete, intense < 10% of tumor cells.

Signature
Dr. Walaa Ghonaim
Dr. Yasmine Essam

Verified by :





Patient Name	: الأستاذة / رشا فريد خلف احمد	Request No.	: 13008015
Patient No.	: 28806012113021	Request Date	: 23/12/2024
Path No.	: 5168/24	Verification Date	: 29/01/2025
Sex	: Female	Financial Category	: اورام دار السلام - نفقة الدولة
Age	: 36 Y	Department	: Out Patient
Referreing Doc.	:	IHC No.	:
		Registered by	: Mrs.Mayada Hegazy

IMMUNOHISTOCHEMISTRY REPORT

Clinical Data: Right breast mass.

Nature of Specimen: Tru-cut biopsy, collected at 23/12/2024.

Technique:

- Sections were prepared from the paraffin block, processed in Bench Mark ULTRA machine (Ventana) and stained by rabbit monoclonal antibodies using ultra view HRP DAB as chromogen and hematoxylin as counter stain. The sections treated against:
- ER clone (1E2), PR clone (SP1), Her2 clone (435). & Ki67 clone (30-9).

Results:

- * IPT (Positive control showed appropriate reactivity):
- ER: Strong nuclear reaction in 80% of tumor cells (score 8/8).
- PR: Strong nuclear reaction in 90% of tumor cells (score 8/8).
- Her2: Moderate complete membranæ reaction in >10% of tumor cells (score 2+).
- Ki67: 5% nuclear reaction of tumor cells.

Conclusion:

Right breast mass, Tru-cut biopsy:

Invasive duct carcinoma, Grade II.

IPT:

- ER: Positive (score 8/8).
- PR: Positive (score 8/8).
- Her2: Equivocal (score 2+) for SISH.
- Ki67: Positive in 5% of tumor cells.

* Scoring system: "ER, PR"

ASCO-CAP ER/PR Test Guideline Recommendations, 2010 Allred score: proportion of cells staining (0,

Verified by :





جمهورية مصر العربية
وزارة الداخلية
مصلحة التفتيش

رشا
فريد خلف احمد
٥٧ ش عبدالعل محمد همام-البراجيل
مركز اوسيم - الجيزة

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ريه منزل
مستلمة
أنش مصطفى ابراهيم عبدالفتاح
مقروجة
البطاقة سارية حتى ٢٠٢٢/٠٢/٢٣

